



COLLEGE
OF
MEDICINE

Papulosquamous diseases

papules

scale

scale papule lesions

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Main objectives

- Highlight on etiology, presentation, complications and treatment of lichen planus.
- Define pityriasis rosea, its etiology, course and management.
- Types of pityriasis rubra pilaris, presentation and treatment.
- Differentiate parapsoriasis from pre-mycosis ~~eruption~~ and management
- What are acute and chronic pityriasis lichenc
- Etiology of erythroderma.



1. Lichen planus

S P disease
↳ synd the fs
↳ Poluritic
Plane (flat top)
Papules
Purplish (Violaceous)
Polygonal (بضلعی)
Polished (برشته دار)

Presentation:

-Typical lesions are intensely itchy, violaceous polygonal, flat topped papules, usually on the extremities, particularly on the volar aspects of the wrists and legs.

↳ inner (Pulse) side of wrist + anterior leg

-Although the lesions are itchy, patients rub rather than scratch so that excoriation are uncommon.

↳ but Palmar aspect of fingers
↳ hence Polished
↳ you can tell Patient do scratch it
to see what there do



- A close look on the lesion reveals white streaky pattern on the surface of the papules (Wickham's ^{scales} striae).
- As in psoriasis Kobner phenomenon may occur.

↳ when an are get affected by trauma the lesion develop at trauma site
↳ seen in many disease like psoriasis or lichen planus





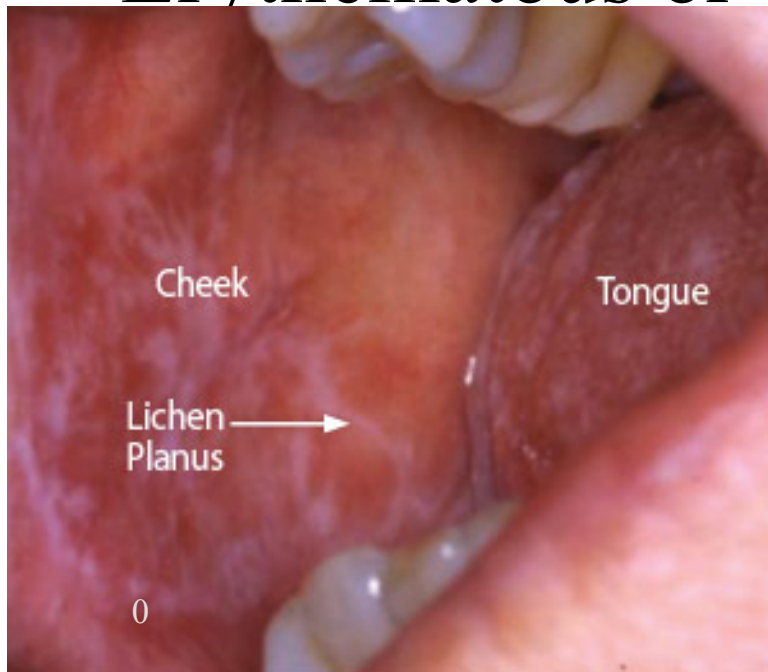


-Oral lesions particularly on the cheeks seen in 50% of cases which may be:

- *Asymptomatic whitish lacy lines, dots and small white plaques (reticulated type)

- *Ulcerative type. *→ may lead to squamous cell carcinoma*

- *Erythematous or erosive type.



- The genital skin may be similarly affected.
- Scalp lesions can cause patchy scarring alopecia.
- The nails are usually normal, but in about 10% of patients show changes ranging from fine longitudinal grooves to entire destruction of nail bed and fold.

called
lichen planus
pillars





Causes:

The precise cause is unknown but: *there are 5 theories*

- a. The disease seems to be mediated immunologically by lymphocytes about the basal cells damaging them.
- b. Graft-versus host disease can cause an eruption rather like lichen planus.
- c. May be associated with autoimmune disorders such as alopecia areata, vitiligo and ulcerative colitis.



d. Drugs like; gold and heavy metals, antimalarials, beta blockers, nonsteroidal anti-inflammatory drugs, para-aminobenzoic acid, thiazide diuretics and penicillamine can cause lichen planus.

e. Some patients with lichen planus also have a hepatitis B or C infection.

So over Lichen Planus Patient screen for HBV & HCV



Variants:

Annular

Linear

Bullous → Blisters

Follicular

Atrophic

Hypertrophic → lesion swelling

Ulcerative → in oral cavity or sole of foot (disabling)



Cause:

- Individual lesions ^{lesions swap each other} may last for many months and the eruption as whole tend to last about one year. ^{to 3 years}
- The hypertrophic variant with thick warty lesions around the ankles lasts for many years.
- As lesions resolve they become darker, flatter and leave brown or grey macules.
- About one in six patients will have recurrence.



Copmlications:

1. Nail and hair loss can be permanent.
2. Squamous cell carcinoma may arise in ulcerative form of mouth.
3. Ulcerative form of skin specially of soles may be disabling.
4. Any association with liver disease which may probably caused by the coexisting hepatitis infection.



Treatment:

Treatment can be difficult, if drugs are suspected as the cause, they should be stopped.

→ for localised
-Potent topical steroids will sometimes relieve symptoms and flatten the plaques.

→ in generalised
-Systemic steroids work too but recommended only in special situations (extensive involvement, nail destruction or painful and erosive oral lichen planus).



-Photochemotherapy with psoraline and ultraviolet A (PUVA) or with narrow band-band UVB may reduce pruritus and help to clear the skin lesions.

→ oral retinoid has to delay pregnancy for 5 years

-Acitretin may help some cases with stubborn lichen planus.

-Antihistamines may blunt the itch.



-Mucous membrane lesions are usually asymptomatic and do not require treatment; if they do, then application of a corticosteroid or tacrolimus in a gel base may be helpful.



2. Pityriasis rosea:

Scale *Red* *Red Scaly*

Cause:

Not known, but human herpes virus 6 and 7 may be responsible. The disease is not contagious.

can also cause Erythema infectiosum (slapped cheek appearance)

Prodrome by upper respiratory system infection

Presentation:

- Common disease especially in winter.
- Mainly affects children and young adults, and second attacks are rare.
- Most patients develop one plaque (herald or mother plaque) before the others. It is larger (2-5 cm in diameter), rounder, redder and more scaly than later lesions.



-After several days many smaller plaques appear, mainly on trunk, but some also on neck and extremities.

-The plaques are oval, salmon pink and show a delicate scaling adherent peripherally as a collarette.

-The longitudinal axis of the lesions run down and out from the spine along the lines of the ribs.

-About half of the patients complain of itching.

-Purpuric lesions are rare.

blood filled lesions



called daughter plaques

smaller more distant

collarette scale (scale at edge) (قشر في الحافة)

like Christmas tree





Course:

- The herald plaque precedes the generalized eruption by several days.
- Subsequent lesions enlarge over first week or two.
- A minority of patients have systemic symptoms such as aching and tiredness.
- The eruption lasts between 2 and 10 weeks and then resolves spontaneously, sometimes hyperpigmented patches fade more slowly.



Treatment:

- No treatment is required.
- A moderately potent topical steroid or calamine lotion will help the itching.
- Sunlight or artificial UVB often relieves pruritus and may hasten resolution.



3. Pityriasis rubra pilaris (PRP) ^{→ red} ^{→ Red scaly hair follicles}

Cause:

- There are several types of PRP, their cause are not known.
- Previously defect in vitamin A metabolism was suggested as a cause, but later disapproved.
- The familial type has an autosomal dominant inheritance.



Presentation:

- The familial type develops gradually in childhood and persists throughout life.
- The acquired type, which is more common, begins in adult life with redness and scaling of the face and scalp. *↪ at hair follicles*
- Later, red or pink areas grow quickly and merge, so that patients with PRP are often erythrodermic. *it red look very close to red color still so all them*



clear non red

Characteristic
Scattered

-Small islands of skin may be spared from this general erythema, but even here the follicles may be red and plugged with keratin.

-Similarly, the generalized plaques, although looks like psoriasis, may also contain follicular plugging.

↳ No follicular plugging



-The palms and soles become thick, smooth, yellow and fissured.

Severe erythema of palms too
↓
Characteristic features



-The acquired form lasts for 6-18 months but may recur.

-When the plaques have gone, the skin may retain a rough scaly texture, with persistent small scattered follicular plugs.





no palm crease
silly white thick scale
no small island
not salmon pink

Differential diagnosis:

Psoriasis is the main differential diagnosis but lacks its slightly orange tinge.

Thickening of palms and soles, follicular erythema in islands of uninvolved skin and follicular plugging within the plaques especially over the knuckles help to separate them.



Treatment:

- Systemic retinoids such as acitretin 25-50mg/day for 6-8 months. *→ Vitamin A*
- Oral methotrexate low dose once a week.
- Topical steroids and keratolytics (2% salicylic acid in soft white paraffin) reduce inflammation and scaling but does not suppress the disorder.
- Systemic steroids are not indicated. *→ No steroid like kawasaki disease*



4. Parapsoriasis and premycotic eruption:

The lesions of this disorder look a little like psoriasis but scale subtly rather than grossly and persist despite anti-psoriasis treatment.



Parapsoriasis



Psoriasis



We should distinguish benign type of parapsoriasis from premycotic type which is forerunner of mycosis fungoides, a cutaneous T-cell lymphoma, however the term premycotic is disputed, as some think that these lesions are mycosis fungoides from the start preferring the term 'patch stage cutaneous T-cell lymphoma'.

Presentation:

Pink scaly well-margined plaques appear on buttocks, breasts, abdomen or flexural skin.



Parapsoriasis (benign type):

- Smaller plaques
- Yellowish
- Sometimes finger-shaped lesions running around the trunk
- No atrophy
- Responds to UVB
- Remains benign although rarely clears.

Premycotic/prelymphomatous eruption:

- Larger
- Not yellow-pink, slightly violet, or brown
- Asymmetrical with bizarre outline
- Atrophy±poikiloderma(atrophy, telangiectasia and reticulate pigmentation) which is most important feature to differentiate the 2 types.
- Responds better to PUVA
- May progress to cutaneous T-cell lymphoma.



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Parapsoriasis (benign type)



Premycotic/prelymphomatous eruption



-Patients with premycotic type should be followed up carefully even though the development of cutaneous T cell lymphoma may not occur for years, if poikiloderma or indurations develops; the diagnosis of cutaneous T cell lymphoma is most likely.





-Several biopsies should be taken for routine histopathological and electron microscopy examination, DNA probes can determine monoclonality of the T cells within the lymphoid infiltrate.

Treatment:

-For benign type: moderately potent steroids or UV radiation, but lesions tend to recur when these are stopped.

-For premycotic type: PUVA or topical nitrogen mustard paint.



5. Pityriasis lichenoides:

Uncommon disease can be precipitated by infection which may precipitate proliferation and skin recruitment of cytotoxic CD8+ T cells. Occur in 2 forms:

1. Pityriasis lichenoides et varioliformis acuta.
2. pityriasis lichenoides chronica.

Similar to chicken pox
→ papule becomes blister → pustule → scarring
↓
Varioform

1. Pityriasis lichenoides et varioliformis acuta:

Characterized by crops of papules that become necrotic and leave scars like those of chickenpox. They rarely affect the face and the rash, which lasts much longer than chickenpox, is usually scattered on the trunk and limbs.





2. pityriasis lichenoides chronica: looks like gutta psoriasis

The numerous small circular scaly macules and papules of the chronic type are easy to confuse with guttate psoriasis. However, their scaling is distinctive in that single silver-grey scales (mica scale) surmount the lesions. The rash usually lasts for months or years.





Erythroderma/ exfoliative dermatitis:

Erythroderma mean the skin is red with little or no scaling, exfoliative dermatitis is preferred if scaling predominate.

Causes:

- Psoriasis
- PRP
- Ichthyosiform erythroderma
- Pemphigus erythematosus
- Contact, atopic
or seborrhoeic dermatitis
- Reiter's syndrome
- Lymphoma
- Drug eruptions
- Crusted scabies







- Most patients have lymphadenopathy and may have hepatomegaly.
- In chronic cases tightness of the facial skin leads to ectropion. *eyelid turned outward away from eyeball*
- Scalp and body hair may be lost.
- The nails become thickened and may shed.
- Temperature regulation is impaired and heat loss through the skin makes the patient feel cold and shiver.



- Edema, high output cardiac failure, tachycardia, anemia, failure to sweat and dehydration can occur.
- Treatment is that of the underlying condition.

